TRAUMA-03 | For therapists | Two separate age-based pathways

Understanding responses and suitable tools

1 | Purpose, fit and boundaries

Purpose of this unit

Help describe current responses, understand the explanation received and record an adapted support tool. Distinguish sensations, thoughts, actions and surroundings without deriving their cause from the sheet.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

No breathing, body or relaxation exercise is mandatory. Reassess unsuitable tools. Do not require distress to disappear before continuing treatment or make a tool compulsory for every encounter with a reminder.

Reusing existing material

REG and SLEEP may support language and habits when suitable; they do not replace trauma-treatment components. Unit 3 focuses on understanding and choice, not processing the event.

A fictional example for discussion

Fictional example: Maya finds focusing on breathing unpleasant. She tells the therapist and they explore another support option within the plan, without inferring she is unready for treatment.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | Age-based pathways

CH / TE | Support within a TF-CBT pathway

Support understanding of responses and emotional language within TF-CBT. Relaxation or regulation tools are chosen separately with the therapist; questions record fit, choice and experience, not technique instructions.

AD / OA | Support within a CPT pathway

Connect the explanation to interpretation, emotion and functioning. Do not add mandatory regulation before CPT or turn child materials into adult treatment instructions.

Parallel work with a suitable support person

Explore how supporters can listen, support and seek help without silencing distress. Choose routine support together and assess their distress separately without making the child soothe them.

Using W and P sheets

Choose the age version and relevant questions; completing every sheet is not a condition for progress. W accompanies professional conversation. P records only pre-agreed follow-up. Unit number does not mean session number.

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

Possible difficulties

When a tool does not help, explore events rather than automatically adding repetitions. New or unusual physical responses need suitable assessment; not every bodily sign is trauma-related.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children, approximately ages 6 to 11

CH uses emotion words the child understands and chosen everyday examples. Supporters can help identify and express needs without ordering calm or dictating answers.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

TE allows for embarrassment, autonomy and differences between home and school. Choose tools with the teenager without requiring family reports of every response.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

AD connects response explanations to meaning and goals in the chosen CPT plan. Support tools do not replace exploring thoughts in treatment or require complete calm beforehand.

OA | Older adults

OA assesses preferences, physical limitations and new changes individually. Offer alternatives or assistance by request, not a uniform exercise based on age assumptions.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

P records support and learning agreed in treatment. It sets no exposure, trauma narrative, dosage or component sequence. Without a plan, bring questions to the session. CH/TE sheets do not replace separate supporter work; AD/OA sheets are not official CPT materials.

At the next session

Check explanation clarity and whether support aids functioning and choice. Immediate tension reduction is not the only measure. Adapt next steps without requiring complete calm.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

Treatment-manual mapping status

Proposed: TF-CBT for children/teens; CPT for adults/older adults. Full manuals remain unmapped. Age, rights, supporter materials and separate-guide needs await review. Original support drafts, not an approved protocol.

www.nctsn.org/interventions/trauma-focused-cognitive-behavioral-therapy
tfcbt.org/about/
www.ptsd.va.gov/professional/treat/txessentials/cpt_for_ptsd_pro.asp

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